

# Siya Health®

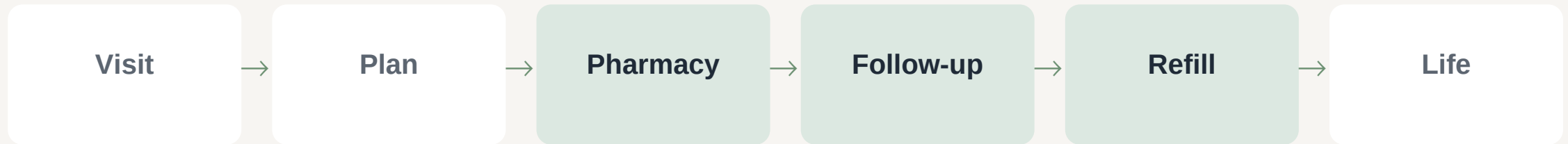
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**In medical school, I watched a brilliant, clever friend struggle for years — distracted, restless, unable to finish what he started — before he was finally diagnosed with ADHD. He's now a successful interventional cardiologist. That gap, between clever-but-struggling and diagnosed-and-thriving, is what we built Siya to close.**

Physician-led ADHD + primary care telehealth · four states (CA · TX · PA · FL)

## The problem

**Patients weren't failing to find care.  
Care was failing to follow them.**



In practice: patients get stalled on records and screening requests before they're ever seen, prescriptions stall on pharmacy shortages, and follow-ups lapse without anyone checking in.

Everyone optimizes the first visit.

**Care actually breaks down at Pharmacy · Follow-up · Refill.**

The solution

# So we built the missing pieces ourselves.

Physicians → Operators → Systems builders

## Offshore clinical care team

Recruited with no prior US healthcare experience; trained on empathetic, plain-language communication and rapid response. New hires reach 5-star patient satisfaction within 6 months. Provider slots filled within 24 hours, across time zones, nights and weekends included. This training pipeline — not just the staff — is the asset: a competitor starting today is 6–12 months behind on replicating it.

## Standardized provider workflows

Protocols and SOPs enforced across every provider and state — checked, not just documented.

## Multi-state licensure

CA · TX · PA · FL — providers credentialed and compliant in every state we serve.

Product

The care journey we run today — end to end.

What we deliver today

End-to-end ADHD care, held together by our protocols and clinical judgment.

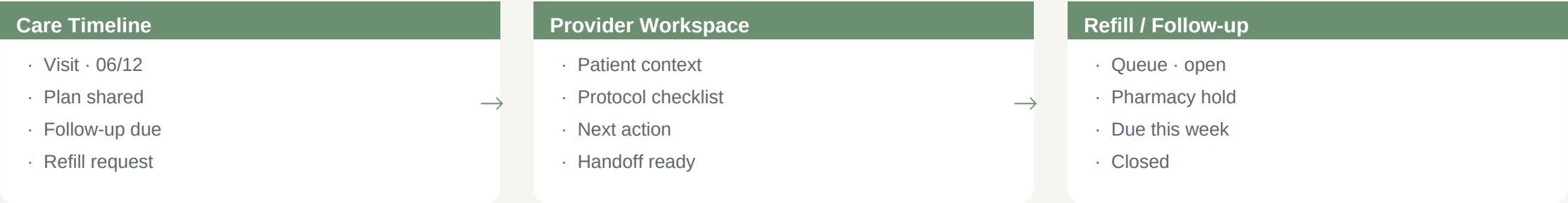


Escalation branch

When care stalls (pharmacy · side effects · missed follow-up · refill risk) → escalate → re-enter Review → Plan → Execute. Continuity is the process.

What we’re building

Concept — in design, informed by workflows already validated in our practice.



Flowchart = care we run today. Wireframe = software encoding that journey (supporting).

Traction

Revenue nearly 3× since February.  
Cash-flow positive.

2,000+

Patients treated

1,000+

ADHD evaluations

150+

Recurring members

600+

Reviews · 4.8★

4 states

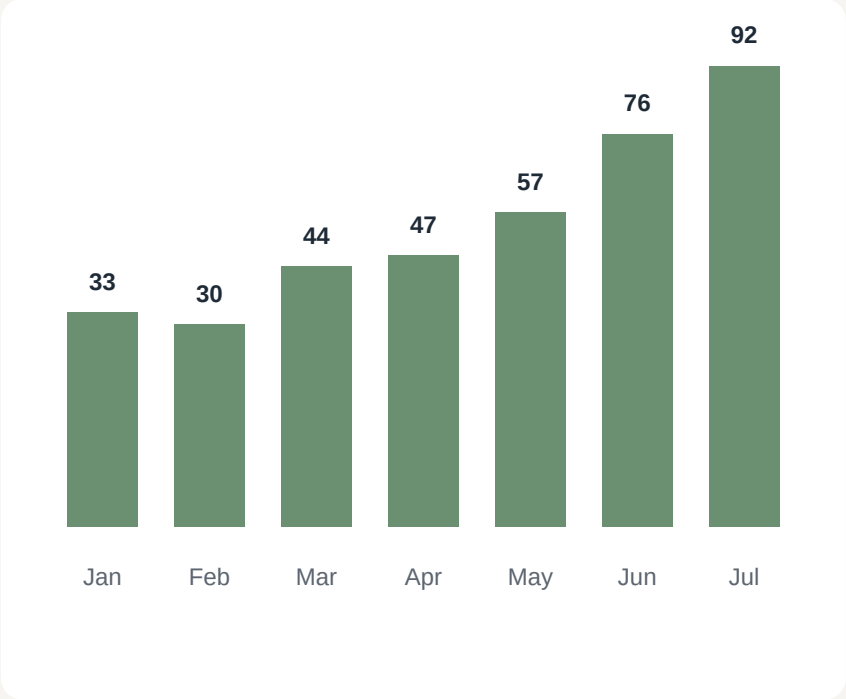
CA · TX · PA · FL

Cash-flow+

Operations

\$378k+ gross 2026 YTD (Jan–Jul) · founder-confirmed books.

Monthly gross revenue — 2026



\$ thousands / month · gross

Where we are, honestly

# Our growth engine today is founder-led — and it works.

## Proof

Founder-led relationships and networking — on top of a full clinical load — fill all providers' panels, not just the founder's own.

## Risk

That is a key-person dependency.  
We say so plainly.

## Fix

This round funds clinical capacity and redundancy, so proven acquisition runs without routing every relationship through one person.

**Proof + risk + fix — not a disclosure buried in an appendix.**

## Cash-pay, recurring, no insurance risk.

**\$149**

**Initial evaluation**

One-time, self-pay

**\$79 / mo**

**Ongoing care — non-controlled**

Recurring membership

**\$149 / mo**

**Ongoing care — controlled**

Recurring membership

Subscription-style continuity: 150+ recurring members and growing.

Roughly \$600 per patient-year in professional fees (bottom-up, competitor-benchmarked).

Cash-pay means no payer contracts, no reimbursement risk, and revenue from day one.

15.5M diagnosed adults. \$2.9B serviceable today.

| Layer | Definition                                                   | Size    |
|-------|--------------------------------------------------------------|---------|
| TAM   | U.S. adult ADHD clinical services (professional fees)        | ~\$10B  |
| SAM   | Adult ADHD virtual care (telehealth-willing, cash-pay comps) | ~\$2.9B |
| SOM   | Year-3, capacity-derived (~40 clinician FTEs)                | ~\$16M  |

Built bottom-up: 15.5M diagnosed U.S. adults (CDC) · ~half already use telehealth for ADHD · ~\$600/patient-year professional fees.

No inflated blended TAM. Full sizing workbook available in diligence.



Competition

Different job. Not a feature checklist.

|                        |                                                         |
|------------------------|---------------------------------------------------------|
| Episodic telehealth    | Optimizes first-visit volume, not continuity            |
| Generic EHR tools      | Records history, doesn't guide next action              |
| Point cognitive tools  | One-time score, not ongoing care                        |
| Traditional psychiatry | Access-limited, waitlist-bound                          |
| Siya                   | Continuity, evaluation through refill — 2,000+ patients |

No feature matrix. No “everyone top-right” quadrant. The difference is the job we do.

## A process — not a channel wishlist.

### 01 · Today

#### Primary engine

Founder-led relationships and networking fill all providers' panels — not just the founder's.

### 02 · Live test

#### Texas Search

Paid Search live in Texas; conversion tracking finalized Aug 2, 2026.

### 03 · Transition

#### This round's job

Add clinical capacity, then move from founder-led fill to systematic acquisition — reproduce Search TX → CA.

### 04 · Measurement

#### No guessing

Full CAC and payback period confirmed by August 30, 2026.

## Why now

# The playbook is proven. The environment rewards how we practice.

### Texas

Early conversions confirmed in Texas — full CAC by Aug 30, 2026.

### California

Revenue growing — next market for the same playbook

### Regulation

Environment rewards protocolized, auditable care over low-touch mills

### Discipline

SOPs, QA, and multi-state compliance already in place

# We're raising \$500k to scale a working practice.

## 01 Clinical capacity & redundancy

More providers and coverage — directly reduces founder key-person risk.

## 02 Scale proven acquisition

Early conversions confirmed in Texas — full CAC by Aug 30, 2026.

## 03 Working capital buffer

Operate from strength, not desperation.

**Objective: 18–24 months of runway to prove the playbook in a second state and remove key-person dependency.**

## Founders

# Physicians who built the practice before asking for capital.



**Dr. Sneh Pandey**

Founder · CEO

- Board-certified physician
- 10,000+ patient encounters at Siya Health across primary care, obesity, hospital medicine, and ADHD care
- Built multi-state practice to 2,000+ patients, cash-flow positive, bootstrapped



**Dr. Swati Pandey**

Co-founder

- Board-certified physician, licensed in Pennsylvania
- Co-founder and clinical advisor — shapes clinical protocol standards across Siya Health

*I was strict about controlled substances in my own training, long before Siya existed. I'd read about companies getting sued for overprescribing, and I didn't want us to become another cautionary tale. So the pill counts, the random drug screens, the compliance formalities — none of that came from a regulation checklist. It came from not wanting my name attached to a company that cut corners on this.*

**Years spent learning how to deliver better care.  
This round scales what already works.**

Seed round

**\$500,000**

Clinical capacity & retention · Leadership capacity · Professional marketing partnership

**Siya Health®**

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**Let's talk.**

**Dr. Sneh Pandey · Founder & CEO**

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